ImaarishaSRHR

PHASE 2 · MEDIA DEVELOPMENT · TOOLKIT 1 OF 3

Strategic Comms & Reframing SRHR

A field guide for Kenyan SRHR organisations: understanding disinformation, culture, patriarchy and the Mother & Aunty frontier — and turning that understanding into evidence-based communication practice.

Community Media Trust (CoMeT) · July 2026

Built on the ImaarishaSRHR Phase 1 evidence base and the wider research literature

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Every figure and case is sourced. Bracketed numbers [n] refer to the Sources list at the end of the document.

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What is in this guide

This is a substantial reference — read a module at a time, or use it as a desk companion when planning a campaign, a community session, or a rapid response. Each module stands on its own.

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HOW TO USE THIS GUIDE

From evidence to practice

For thirty years, SRHR programming in Kenya has expanded services and produced accurate information — and yet 355 women still die for every 100,000 live births, roughly a third of those deaths from unsafe abortion, and about half of women who use contraception do so in secret. The Phase 1 research asked why, and turned the critical lens inward. Its answer reframes the whole problem: the binding barriers are not mainly about clinics, distance or knowledge. They are about meaning, trust, power and fear — about who gets to define what reproductive health means in public and private life.[1,2]

This guide converts that finding, and the wider global evidence on norm change and health communication, into something an SRHR organisation can actually use. It is deliberately substantial: each module gives you the theory, the Kenyan data, real documented cases, and practical tools — enough to brief a new staff member, design a campaign, or defend a strategic choice to a funder.

The central argument, in one paragraph

> Kenyans do not experience SRHR as a rights issue. Only **8%** associate it with rights and **2%** with wellbeing; most think first of clinics and contraception. Meanwhile fear of judgment (**76%**) and stigma (**72%**) outrank cost, distance and availability as barriers, and only **3.6%** of online SRHR content is affirming. The response the evidence points to is not louder rights advocacy. It is **values-based communication, carried by trusted messengers, that meets people where they are and moves them from there** — health as the entry point, family wellbeing as the door, rights as the destination.

How each module is built

The story. Each module opens on a scene from one recurring Kenyan family — the Achiengs — so the concept arrives inside a moment you can picture, and your illustrators can draw. Meet them in "The cast," next.

The evidence. What the Phase 1 research and the wider literature establish — with exact figures and sources.

The frameworks. The theory that explains the evidence — social-norms theory, the gender continuum, framing theory, the infodemic model, diffusion of innovations — so your team can reason, not just follow rules.

Case in point. Real, documented cases — Kenyan wherever possible — that show the evidence at work.

Entry points for your organisation. A green box in every module translating the scene into concrete things your organisation can do — the practical hooks.

Practical tools. Frameworks applied: message formulas, diagnostic questions, checklists and templates.

For your practice. A short read-across you can fold into existing routines.

A note on the evidence — and its honesty

This guide does not oversell. Where the evidence is mixed it says so — a well-funded programme like DREAMS did not, by itself, accelerate the decline in HIV incidence in one rigorous Kenyan cohort, and dialogue-only interventions have documented limits without structural change and male engagement. Reliable communication is built on honest evidence, not promotional evidence.[6]

THE EVIDENCE BASE

Where this guide comes from

The Phase 1 research triangulated three independent methods so that findings are reported as confirmed only where at least two methods converge. That standard of evidence is what gives this guide its authority — and it is worth understanding before you use the numbers.

Three methods, one triangulated truth

Method	What it captured	Scale & scope
Opinion poll (TIFA)	What Kenyans nationally think SRHR means, believe about it, and who they trust — the macro picture.	n=2,013 · all 47 counties · ±2.18% · face-to-face · May 2026
Social listening	What the public SRHR conversation actually looks like online — narratives, sentiment, disinformation, reach.	419,720 posts · Jul 2025–Feb 2026 · 93% on X · English, Kiswahili, Sheng
WhatsApp focus groups	Lived experience in women's own words — the meso/micro texture behind the numbers.	Women 18–35 · 4 sites: Nairobi (2), Narok, Homa Bay · 10–15 per group

Study designs and samples: [3,4,5,2].

The seven narrative streams — the analytical spine

The social-listening analysis organised Kenya's entire SRHR conversation into seven recurring "narrative streams." This guide uses them as connective tissue: each module maps to one or more streams, and the streams' documented interactions are the evidence for Module 1's central claim that these forces are one system. [2]

Stream	What it carries
S1 · Health, Services & Rights	Medical, service-delivery and rights-based framing — the sector's own language.
S2 · Moral & Religious	Sin, moral policing, faith-based mobilisation.
S3 · Cultural & National Identity	SRHR framed as "un-African," foreign-imposed, a threat to tradition.
S4 · Patriarchy & Male Authority	Male dominance, gendered digital voice, reproductive control.
S5 · Generational & Social Control	Shame language, intergenerational norm enforcement, TikTok as the youth classroom.
S6 · Disinformation & Conspiracy	Fertility myths, HIV faith-healing, anti-CSE rhetoric.
S7 · Policy & Political Process	Legislation, supply-chain failures, the advocacy hashtag ecosystem.

The national context, in six numbers

355 maternal deaths per 100,000 live births (KDHS 2022)	**~35%** of maternal deaths linked to unsafe abortion	**82** births per 1,000 girls aged 15–19
41% of new HIV infections are among girls & young women 15–24	**34.5%** unmet family-planning need among unmarried women 15–19	**Art. 43** Constitution guarantees the right to reproductive health

National context figures cited in the Phase 1 report from KDHS 2022 and the National Syndemic Diseases Control Council [2]; MMR and unsafe-abortion share corroborated by KDHS 2022 and Center for Reproductive Rights.

THE AUDIENCES

Who Phase 2 must reach

Communication is only as good as its fit to a specific audience. The research identifies four priority audiences, each needing a different frame, messenger and channel. Every module returns to them; this table is the map.

Audience	Why they matter	What the evidence says
Women 18–35	The primary decision-makers whose access the whole effort exists to widen.	Half use contraception secretly; fear of judgment (76%) and stigma (72%) are their top barriers.
Men as partners	They hold disproportionate decision authority — and most resistance is correctable.	Misinformation is the leading reason they resist (45%); shared-decision framing moves them (53%).
Older women (mothers, aunties)	The most trusted channel — and the least engaged. Phase 2's highest-leverage point.	67% shape younger women's confidence; the channel can carry accurate scripts instead of myths.
Youth 18–24	Forming beliefs and making first decisions, online and information-poor.	Get first SRHR information from peers/social media; Sheng content outperforms English ~3:1.

Audience evidence: TIFA opinion poll and the ImaarishaSRHR research report [3,2].

A fifth cross-cutting audience — faith and community leaders — is addressed throughout as both the sharpest risk (Module 2) and, engaged well, the strongest ally (Module 6). Hold all of them in view: the system is connected, and so must the response be. [2]

THE CAST

One family carries the whole guide

This guide is built around a single extended Kenyan family — the Achieng family of a Nairobi peri-urban estate, with roots in Homa Bay — and the community around them. Their world runs through every module, so the frameworks are never abstract: each concept arrives inside a scene you can picture, from a specific person, in a specific kitchen or WhatsApp group. This is the entertainment-education principle applied to a toolkit — we learn through people we come to know, not through bullet points. Illustrators should treat the descriptions below as the character bible: the same faces recur across the toolkit, the videos and the podcast, so audiences come to recognise them. [2]

The Achieng family & their community

Character	Who they are	What they carry (narrative stream)
Agneta, 17	Form 3 student, wants to train as a nurse; bright, anxious, petrified that a pregnancy will end her education. The young woman at the centre.	S5 Generational control · the youth cohort
Aunty Beth, 52	Agneta's paternal aunt (her father's sister); runs the household kitchen; respected, warm, admin of the "Achieng Family" WhatsApp group. The gatekeeper who can become a champion.	S5 · S4 · S6 — the Mother & Aunty frontier
Mama Grace, 44	Agneta's mother; loves her daughter, defers to her husband and to Beth; uses contraception secretly to space her own births.	S4 Patriarchy — constrained agency
Baba Tom, 50	Agneta's father; holds household decision authority; believes a big family proves a man; follows "#MasculinitySaturday" content.	S4 Patriarchy & male authority
Uncle Silas, 39	Tom's younger brother; kind, devout, forwards every dramatic health "warning" to the family group — HIV cures, contraceptive scares.	S6 Disinformation · S2 Moral & religious
Rev. Otieno, 55	The family's pastor; his Sunday word shapes what the family believes is right — a risk, and, engaged well, the family's strongest ally.	S2 Moral & religious · S3 Cultural identity
Nurse Wanjiru, 34	Clinical officer at the local health centre; runs a youth-friendly corner; the most trusted voice in the story, but overworked and under-heard.	S1 Health & rights — the Level 2 messenger
Kevo, 22	Agneta's cousin; a Sheng-speaking TikTok creator with a real following; the bridge to the digital street.	S5 · the youth digital classroom
Brenda, 19	Agneta's friend who left school after a pregnancy; now a peer advocate telling her own story.	Survivor testimony · Bright spots
Cucu (Grandmother)	The family matriarch in Homa Bay; carries the deepest traditional norms; the apex of intergenerational authority.	S5 · S3 — the rural, offline frontier

Characters are composites built from the Phase 1 focus groups, social listening and opinion poll — not real individuals [5,4,3].

How the family maps to the modules

Each module opens on a scene from this family and returns to it. The same event — Agneta's fear of a pregnancy derailing her future — is seen from a different angle in each module, so the modules interlock into one story rather than seven separate lessons. [2]

Module	The anchoring scene	Whose story leads
1 · The System	One girl, four forces: Agneta's fear pressed at once by her father, the pulpit, a WhatsApp forward, and a backlash.	Agneta (all)
2 · Disinformation	The family WhatsApp group lights up: Uncle Silas's "HIV cured" forward and Beth's "Depo makes you barren."	Uncle Silas & Beth
3 · Culture & Patriarchy	Baba Tom decides the family size and finds Mama Grace's hidden pills; the cost of a woman's choice.	Mama Grace & Tom
4 · Mother & Aunty	The kitchen: Agneta confides her fear to Beth while they cook — and Beth's "marry at 20" advice.	Beth & Agneta
5 · Reframing	The same facts, rebuilt to reach Tom (provision) and Beth (protecting Agneta), voiced by Nurse Wanjiru.	Nurse Wanjiru
6 · What Works	The corner that changes things: Wanjiru's youth desk, Kevo's Sheng clip, Rev. Otieno turned ally, Brenda's story.	The whole community

ILLUSTRATION *Establish the family in one warm group scene — the estate compound at dusk, Beth at the sufuria, Agneta beside her, Tom and Silas on plastic chairs with a phone between them, a framed photo of Cucu on the wall. This is the recurring world; keep faces, clothing and the kitchen consistent across all modules and the video slate.*

A note on respect. No one in this family is a villain. Beth, Silas, Tom and Cucu all act out of love and duty — that is precisely why the harm they pass on is so hard to counter, and why the response is never to shame them. The story's arc is not defeat but conversion: Beth, by the end, becomes the family's strongest champion for Agneta's health.[2]

MODULE 1

01 The System

The question this module answers: *Why do our best single-issue campaigns get somewhere — and then stall?*

IN THIS MODULE

- The core claim: patriarchy, culture/religion and disinformation are one machine, not three problems
- Systems thinking and the social-ecological model
- The four gears: how each force powers the others
- The documented cross-stream interactions in Kenya's SRHR conversation
- Two Kenyan cases: the Marie Stopes ban and #ScienceBows
- Tool: Map the Machine — a design diagnostic

THE STORY · MODULE 1 • **One girl, four forces**

Follow one ordinary fear through one ordinary week. Agneta, seventeen, wants to be a nurse and is terrified a pregnancy will end that. Watch what presses on her — not one obstacle, but four, all at once.

Monday, the kitchen. Aunty Beth tells her family planning will ruin her womb; marry at twenty instead. (Culture & the intergenerational channel.)

Wednesday, the phone. Uncle Silas forwards a clip to the family WhatsApp group: contraception causes cancer. It arrives with a prayer emoji. (Disinformation.)

Friday, the estate wall. A new NGO poster: "It's your body, your right to choose." Baba Tom sees it, mutters that the world is teaching his daughter disobedience, and forbids any clinic talk at home. (Patriarchy — and backlash.)

Sunday, the pulpit. Rev. Otieno preaches that family planning is tampering with God's plan. Half the congregation nods. (Moral authority.)

By Monday, Agneta has more accurate information than her mother's generation ever had — and less freedom to act on it. No single message could have reached her, because no single force was holding her.

NARRATIVE STREAMS S2 Moral & religious · S3 Cultural identity · S4 Patriarchy · S6 Disinformation · S7 Backlash

The evidence: four forces, one machine

The single most important analytical shift in the Phase 1 research is this: patriarchy, culture and religion, and disinformation do not operate as separate barriers that can be tackled one at a time. They form a self-reinforcing system in which each force lends the others its power. The social-listening analysis did not merely observe the four forces side by side — it documented them actively reinforcing one another across the seven narrative streams. This is why interventions that address a single dimension consistently get somewhere and then stop. [2,4]

One number captures the stakes. Across 278,770 primary posts, only 3.6% of SRHR content carried positive, affirming or empowering sentiment; 29% was actively negative and the remaining two-thirds neutral. That 3.6% is not just a gap in the conversation — it is space the sector has left open, into which the connected system of patriarchy, culture and disinformation has expanded. [4]

3.6%	29%	2:1
of digital SRHR content is positive or affirming	is actively negative — fear, stigma, moral conflict	men outnumber women in the public SRHR conversation

Sentiment and participation: ImaarishaSRHR social listening [4].

The frameworks that explain it

FRAMEWORK · **The social-ecological model — barriers live at four levels**

Public-health practice locates the drivers of behaviour at four nested levels: the **individual** (knowledge, beliefs), the **interpersonal** (partners, family, peers), the **community and institutional** (norms, providers, faith bodies, media), and the **structural / policy** (law, supply chains, the economy). A systematic review of adolescent SRHR in sub-Saharan Africa found that barriers cluster overwhelmingly at the community and structural levels — not the individual one. This is the academic form of the Phase 1 finding: "low demand" is usually a structural condition wearing an individual disguise. [7]

FRAMEWORK · **The self-reinforcing "Triple Threat" — the four gears**

Read the system as four gears that turn each other:

Moral authority legitimises disinformation. Faith and cultural authority lend fertility myths and HIV-cure claims a credibility that armours them against factual rebuttal.

Cultural identity covers patriarchal control. Male reproductive authority is repackaged as "cultural duty," so challenging it reads as an attack on identity, not on one belief.

Intergenerational channels carry the myths. Contraceptive misinformation travels through mothers and aunties — the most trusted people — delivered as love.

Policy visibility generates backlash. Rights-based advocacy can trigger organised opposition that outpaces it in reach and emotional resonance. [2]

The proof: documented cross-stream interactions

The claim that these forces are one system is not rhetorical. The social-listening analysis confirmed specific, repeatable interactions between the seven narrative streams — each an instance of one force powering another. [4]

Interaction	What it looks like	Evidence signal
S2 + S6 Moral authority × Disinformation	Faith actors give HIV-cure and contraceptive myths religious credibility.	#ScienceBows: 16.8M reach, 100/100 positive sentiment among followers
S3 + S4 Cultural identity × Patriarchy	Anti-CSE opposition frames male reproductive authority as cultural duty.	CSE reframed as "foreign values," activating identity not evidence
S5 + S6 Generational control × Disinformation	Contraceptive myths travel through trusted family relationships.	"Depo makes you barren," carried by mothers/aunties, hardest to dislodge
S7 + S2 Policy visibility × Moral backlash	Rights advocacy generates visibility but triggers organised opposition.	#DefendThePromise: bottom sentiment outlier despite high visibility

Cross-stream interactions and hashtag metrics: ImaarishaSRHR social listening [4].

CASE IN POINT • **The Marie Stopes ban (2018) — culture, faith and policy converging**

In late 2018 the Kenya Medical Practitioners and Dentists Board ordered Marie Stopes Kenya to stop all abortion-related services, after an anti-abortion complaint about its media campaigns; the Kenya Film Classification Board had already ordered the adverts pulled. The restriction was later challenged in court and lifted.

This is the machine in miniature. Moral and faith-aligned objection (Streams S2/S3) converted disapproval into a regulatory decision (S7) that removed a service millions relied on — even though roughly a third of Kenya's maternal deaths are linked to unsafe abortion. A single-issue, rights-first defence was structurally exposed because the opposing forces were connected: religious authority, cultural framing and policy machinery moved together.[8,9]

CASE IN POINT • **#ScienceBows — when moral authority legitimises disinformation**

Over eight weeks in early 2026, a claim that HIV could be cured by divine intervention moved from a single broadcast to a national risk. It generated 140,950 posts — roughly half the volume of the entire preceding six-month study period — carried by a faith network whose lead account reaches over half a million people per post. #ScienceBows alone drew 6,957 mentions (+131% above median), an estimated 16.8 million reach, and 100/100 positive sentiment among followers; the Ministry of Health was forced to investigate, and community organisations reported people living with HIV stopping their ARVs.

No single gear explains this. Religious authority (S2) gave a medical falsehood (S6) emotional armour; the message travelled through trusted intimate networks (S5); and it outran the slow institutional correction (S7). A fact-check alone never stood a chance.[4,2]

Why single-dimension interventions underperform

If the barrier is a system, an intervention that touches only one part will be neutralised by the others. Distribute accurate leaflets, and the mother's loving warning still outweighs them. Win a policy, and the backlash outreaches the win. Train one clinician, and community stigma still keeps women from the door. The evidence for this is the pattern of near-misses: campaigns that generate visibility without sentiment change, and services that exist but go unused. Disruption, to hold, has to be systemic — hitting more than one gear at once.[2]

The design implication is concrete. The strongest interventions in the Kenyan evidence combine an **accurate message** (counters disinformation), delivered by a **trusted messenger** such as a clinician or a respected older woman (uses the intergenerational channel), **framed around health and family wellbeing** (defuses the identity trigger). One piece of content; three gears.

A closer look: the same power imbalance runs online

The system is not confined to households and clinics; it reproduces itself in the digital public square. In the social-listening dataset, men authored roughly twice as many SRHR posts as women (6,425 vs 3,237 — 20.3% against 10.2% of authored content) and carried a higher share of negativity (32.8% of male posts vs 26.8% of female). Women became visible mainly at moments of direct bodily threat — HIV, abortion, GBV — while men set the adversarial, judgment-driven terms of the everyday conversation. The offline architecture of male authority is mirrored, almost exactly, in who speaks and how online.[4]

Even the vocabulary reveals it. Men's SRHR language in the dataset was protective and self-referential — *"rubbers," "mpira," "tying the pipe"* — framing reproduction as something managed from a distance rather than

shared. Communication that ignores this gendered speech economy will, by default, speak to men in women's terms and reach neither.[4]

> *"Men decide when to have children and how many."*
> — Female, 18–35 · Narok focus group[5]

"Constrained agency": what the system does to choice

The research names the lived result of the system precisely: constrained agency within unequal power systems. Women's agency is real — but it is exercised covertly, under threat of conflict, rejection or violence. Roughly half of women who use contraception do so secretly; disclosure is delayed; choices are navigated around a partner rather than with one. This matters for communication because it changes the job to be done: the task is not only to inform a woman of her options but to reduce the real-world cost of acting on them — a cost the system, not the individual, imposes.[2,5]

> *"Fear of separation makes women avoid asserting their choices."*
> — Female, 18–35 · Nairobi informal settlements focus group[5]

This is the bridge to the rest of the guide. If the barrier is a connected system that imposes a cost on women's choices and reproduces itself in trusted relationships and online, then the response cannot be a single message, a single channel, or a single messenger. It must be **systemic, trusted and reframed** — the three ideas that organise Modules 2 through 6.

A walk-through: one barrier, four gears

To make the system concrete, trace a single common barrier — "a young married woman wants to space her births but does not use contraception" — through all four gears. Seeing the whole machine at once is what stops an organisation from investing everything in one gear and wondering why the barrier holds.[2]

- **Patriarchy.** Her husband holds decision authority; she fears his reaction, so any use must be covert — the injectable is chosen because it hides. The barrier is enforcement, not her ignorance.
- **Culture & religion.** The pastor frames family planning as interfering with God's plan; her mother-in-law warns that it invites infidelity. Her choice now carries moral and spiritual cost, not just marital cost.
- **Disinformation.** A WhatsApp forward and a TikTok comment insist the injectable causes permanent barrenness — a fear that lands harder than any leaflet reassures.
- **Backlash / visibility.** A rights-framed clinic poster that says "it's your body, your choice" is read by her husband as the outside world encouraging disobedience, and hardens his opposition.

Any single intervention — a leaflet, a poster, one clinic visit — is neutralised by the other three gears. A message designed for the system looks different: a trusted older woman or CHP (works the intergenerational and patriarchal gears), carrying an accurate, WhatsApp-shareable correction about fertility return (disinformation gear), framed as "planning a healthy family together" rather than rights (defuses the backlash gear). That is one piece of content touching three gears — the standard this guide holds every message to.[2]

Tool: Map the Machine

Use this diagnostic when a barrier feels stuck. Take one real problem your organisation faces — "women hide contraception," "men throw away pills," "young people believe Depo causes barrenness" — and work the four questions. The output is a one-page system map that feeds directly into message design (Module 5).

1. **Name the forces.** For each of patriarchy, culture/religion, and disinformation, write one sentence: how does this force feed the barrier?
2. **Trace the reinforcement.** Draw the arrows between the forces, not just to the barrier — where does moral authority armour a myth, or identity cover control?
3. **Find the shared channel.** Who is the trusted messenger the system currently uses to carry this barrier — and could that same channel carry a different message?
4. **Design for two gears.** Sketch one intervention that touches at least two forces at once. If it touches only one, expect it to stall.

ENTRY POINTS FOR YOUR ORGANISATION • **Designing for Agneta's whole week**

→ **Map the forces before you design.** For any campaign, run the Map the Machine tool below on a real young person like Agneta — never design against one force in isolation.

→ **Build one asset that touches two gears.** E.g., a WhatsApp voice note from a champion aunty (intergenerational + patriarchal gears) carrying an accurate fertility fact (disinformation gear), framed around family (backlash gear).

→ **Coordinate, don't compete.** The forces are connected and well-resourced; a single small organisation cannot out-shout them alone. Plan reach as a coalition.

FOR YOUR PRACTICE • **Building the system view into your work**

→ Add a one-line "which gears does this touch?" check to your campaign and activity planning templates.

→ Before approving any SRHR message, ask whether it accidentally triggers the identity or backlash gear.

→ Keep your system maps visible in the planning space and update them as you learn what moves.

MODULE 2

02 Understanding Disinformation

The question this module answers: *Why can't we simply fact-check our way out of this?*

IN THIS MODULE

- The scale of Kenya's SRHR infodemic
- Why corrections fail: the continued-influence effect
- The four disinformation typologies, in depth
- Anatomy of a crisis: #ScienceBows, week by week
- A precedent: the 2014 tetanus-vaccine boycott
- The science of countering: prebunking and the truth sandwich
- Tools: the rapid-response protocol and the counter-message formula

THE STORY · MODULE 2 · **The night the family group lit up**

The "Achieng Family" WhatsApp group has 34 members and one admin: Aunty Beth. Most days it carries greetings, prayer chains and photos. Tonight it carries something else.

Neither Silas nor Beth is lying, in their own minds. Each is passing on what they received as truth, to people they love, on the most trusted channel they have. By morning the "HIV cured" clip has left the family group and reached three other groups. This is where Kenya's SRHR disinformation actually lives — not on some distant platform, but in the family chat, wearing the face of love.

ILLUSTRATION *Two phone screens side by side in a dark room — the green WhatsApp glow on Silas's and Beth's faces. Show the messages as chat bubbles. The warmth of the family group is the point: this is not a troll farm, it is home.*

NARRATIVE STREAMS **S6 Disinformation & Conspiracy · S2 Moral & religious · S5 Generational control**

The evidence: an information environment tilted against health

Kenya's SRHR information environment does not merely contain some falsehoods — it is structurally tilted against accurate content. Only 3.6% of monitored posts were affirming; abortion discourse ran at a 15:1 negative-to-positive ratio, with zero positive posts among its ten most visible authors. Disinformation also out-reaches prevention: at the height of the 2026 crisis, HIV-cure content reached an estimated 16.8 million people while the parallel International Condom Day messaging reached 2.35 million — a reach disadvantage of more than 7:1 for accurate prevention communication. Practitioners at the stakeholders' forum reported spending more time demystifying false information than delivering services. [4,2]

15:1	140,950	7:1
negative-to-positive ratio on abortion — highest in the dataset	posts in the 8-week HIV faith-healing wave	reach advantage of HIV-cure disinfo over condom-day messaging

Infodemic metrics: ImaarishaSRHR social listening [4].

Why you cannot fact-check your way out

FRAMEWORK · **The continued-influence effect — why a myth survives its correction**

Decades of cognitive-science research show that once misinformation is encoded, simply labelling it "false" rarely erases its influence: people continue to rely on the original claim even after they accept the correction. Corrections work best when they do not just negate the myth but replace it with an equally clear, causal alternative account — otherwise the correction leaves a gap the myth rushes back to fill. This is why "myth-busting" that repeats the myth loudly and adds a small "FALSE" stamp can backfire, and why the source of a correction matters as much as its content.

Practical rule: never lead with the myth. Lead with the true, memorable fact; state the myth once, clearly labelled; explain the manipulation or error; then repeat the fact. (The "fact–myth–fallacy–fact" structure, below.) [10]

FRAMEWORK · **Prebunking (inoculation) — building resistance before the myth arrives**

Inoculation theory shows that briefly exposing people to a weakened form of a manipulation technique — and explaining how it works — builds durable resistance to it, much as a vaccine primes the immune system. Field and online experiments find that short "prebunking" messages improve people's ability to spot manipulation and reduce their likelihood of sharing it. For SRHR, this means teaching audiences the tell-tale moves of health disinformation (a personal "miracle" testimony, borrowed medical-sounding language, an urgent call to stop treatment) before the next wave hits — not only rebutting each claim after it spreads. [11]

The four disinformation typologies

The social-listening analysis classified Kenya's SRHR misinformation into four recurring typologies. Each requires a different response, and each is powerful for a different reason. [4]

Typology	What it claims	Why it sticks	Data signal
1 · Contraceptive safety myths	"Depo makes you barren." "The pill damages your womb." "Birth control causes cancer."	Carried by mothers, aunties and WhatsApp as protective warnings — love, received as truth. The most personally consequential typology.	47% of Kenyans believe contraception can cause infertility
2 · Fertility & abortion fears	"Abortion permanently destroys the womb." "Termination causes lifelong infertility."	A pre-adjudicated moral verdict masquerading as health information; abortion enters the conversation as a settled verdict.	15:1 negative; zero positive posts among top 10 abortion authors
3 · Anti-CSE rhetoric	"CSE teaches promiscuity and homosexuality." "Foreign values imposed on our children."	An authority contest, not an evidence dispute; activates nationalist, religious and parental fear at once, framed as protection.	Sustained, institutionally backed campaigns
4 · HIV faith-healing & ARV distrust	"HIV can be cured by divine intervention." "ARVs are unnecessary or harmful."	Carries trusted religious authority; live-stream testimony borrows scientific legitimacy. The most acute current risk.	#ScienceBows: 16.8M reach; MoH forced to investigate

Typologies and belief data: ImaarishaSRHR social listening [4]; contraceptive-infertility belief (47%): TIFA opinion poll

[3]

Anatomy of a crisis: #ScienceBows, week by week

The 2026 HIV faith-healing wave is the template every SRHR organisation should study, because it shows how a single claim becomes a public-health emergency at social speed — while the institutional correction crawls behind. [2,4]

Weeks	What happened	Where a response could have interrupted it
1–2	A claim that HIV can be cured by divine intervention is broadcast to a faith network whose lead account reaches ~540,000 per post; the clip hits WhatsApp.	Pre-existing faith-leader alliances; a rapid clip-level rebuttal from a trusted, named clinician.
3–4	#ScienceBows and #HIVHealedMustWatch trend; the "Rebecca Mose" testimony spreads with medical-sounding language; WhatsApp groups amplify.	Same-channel counter-content (WhatsApp-native, doctor-voiced, shareable); a prebunk on the "miracle testimony" pattern.
5–6	~16.8M reach; 100/100 positive sentiment among followers; people living with HIV reportedly stop ARVs; clinical cases documented.	A coordinated sector response and survivor/clinician testimony at equivalent emotional pitch.
7–8	The Ministry of Health is forced to investigate and issues guidance — after the wave has peaked.	A pre-agreed escalation protocol and faster institutional voice.

Timeline and metrics: ImaarishaSRHR social listening and Insight Report [4,2].

The mechanics repay close reading. The wave worked by stacking four credibility signals: a **personal testimony** ("Rebecca Mose is no longer HIV positive") presented as verified proof; **borrowed medical language** ("doctors testify to medically verified outcomes"); **religious authority** broadcasting at scale; and a **live-stream format** whose very medium signalled credibility. Recognising this stack is the basis of prebunking it. [4]

CASE IN POINT · **A precedent ignored: the 2014 tetanus-vaccine boycott**

#ScienceBows was not without warning. In 2014, the Kenya Conference of Catholic Bishops publicly claimed that a WHO/UNICEF tetanus-vaccination campaign for women of child-bearing age was a covert mass-sterilisation programme laced with the hCG hormone. WHO and UNICEF denied it and independent testing did not support the claim, but the announcement — carried by trusted religious authority — depressed vaccine uptake and took months of counter-communication to contain.

The parallel is exact: a health intervention, recast through faith authority as a hidden attack on fertility, spreading faster than the correction. The lesson the sector did not fully absorb in 2014, and paid for in 2026, is that pre-emptive alliances with faith leaders — before a crisis — are the only reliable prevention. [12]

CASE IN POINT · **The long campaign against sexuality education (Typology 3 in the wild)**

Anti-CSE content is not a passing rumour; it is a sustained, organised campaign. The Kenya Conference of Catholic Bishops and allied groups have repeatedly campaigned against comprehensive sexuality education, framing it as "foreign values" imposed on African families — a framing that activates religious, nationalist and parental fears at once and shapes what schools are willing to teach.

Because Typology 3 is backed by durable institutions with wide reach, isolated fact-checks rarely move it. It is contested on the terrain of identity and authority — which is why coalition, trusted-messenger engagement and reframing matter more here than rebuttal. [13,14]

The science of countering: three practical rules

> FRAMEWORK · **Fact — Myth — Fallacy — Fact (the "truth sandwich")**
>
> The evidence-based structure for a correction is: **lead with the true fact** (clear and memorable) → **warn of the myth once** ("some people claim…", stated a single time) → **expose the fallacy** (name the manipulation: a fake testimony, a false authority) → **repeat the fact**. The myth is sandwiched between two statements of truth so the audience leaves with the fact on top, not the falsehood. [15]

The three rules in one line each

- **Lead with truth, not the myth.** Never make the false claim the headline; it is what people remember.
- **Match the channel and the messenger.** A correction on a different platform, in an institutional voice, does not reach the people who saw the myth. Use the same channel and a trusted human (a named clinician, 71% trust).
- **Carry equal emotional weight.** Disinformation travels on fear and hope. A dry, technical rebuttal loses to a warm, confident one.

Worked example: rebuilding a real counter-message

Take the single most consequential myth in Kenya — that contraception causes infertility, believed by 47% of adults. Here is the same correction written the wrong way and the right way.

The way that backfires	The truth-sandwich version
"It's FALSE that family planning makes you barren! Stop believing this myth!" (Leads with the myth, repeats it, shames the believer, no alternative account, no source.)	FACT: "When you're ready to have children, modern family planning lets your body return to normal." MYTH (once): "Some people warn that it makes you barren." FALLACY: "That warning is passed on out of love, but it isn't medically true." FACT: "Your fertility comes back after you stop — a health worker can show you how each method works."

The right-hand version leads with a memorable truth, states the myth only once, names why it spreads (honouring the loving intent — the Module 4 move), supplies the causal alternative the mind needs, and points to a trusted source. It is also short enough to forward on WhatsApp. Notice what it does ***not*** do: it never makes the false claim the headline. [15]

Where the risk concentrates

Disinformation is not evenly spread, and knowing where it concentrates lets a small team target scarce effort. Two patterns stand out. First, negativity is geographically uneven: Eastern Kenya carried the highest negative share of any region (39.0%), making it the most disinformation-vulnerable, while Nyanza carried the highest positive share (5.0%) — a base to reinforce. Second, the most normatively intense communities are the least visible online: Narok and Homa Bay are almost absent from the digital conversation despite intense offline barriers. Digital silence is not the absence of need; a digital-only strategy would miss exactly the communities that need most. [4]

Region	Share of digital voice	Negative share	What it means for you
Nairobi	71.2%	34.5%	Where digital strategy and rebuttal must live; not representative of the country

Region	Share of digital voice	Negative share	What it means for you
Eastern Kenya	0.7%	39.0% (highest)	Most disinformation-vulnerable — prioritise prebunking
Nyanza	1.7%	32.9% (5% positive)	Highest positive share — reinforce and scale
Narok / Homa Bay	Near-absent	Intense offline	Community-first, radio and CHP — digital alone will miss them

Regional distribution: ImaarishaSRHR social listening [4].

Prebunking in practice: the four tell-tale signs

Because prebunking teaches people to recognise the technique rather than memorise every false claim, it scales in a way rebuttal cannot. Teach your audiences — and your own staff and CHPs — to notice these four moves, which recurred across Kenya's SRHR disinformation. Naming the move is often enough to break its spell. [4]

- **The miracle testimony.** A single dramatic personal story ("she is no longer HIV positive") offered as proof for everyone. One story is not evidence.
- **Borrowed authority.** Medical-sounding language or vague "doctors confirm" claims attached to a non-medical source.
- **The urgent instruction.** A push to act immediately — stop your treatment, refuse the method — before you can check. Urgency is a manipulation tell.
- **The identity hook.** "Real Africans / true believers / good parents reject this." When a health claim is really about belonging, be sceptical.

The WhatsApp problem

Much of Kenya's most consequential SRHR misinformation travels not on open platforms but in closed WhatsApp groups and family chats — the same intimate Level 1 channel that carries a mother's warning (Module 4). This has two implications. First, you often cannot see it spreading, which is why the listening rota must include trusted community informants, not only public-platform monitoring. Second, you cannot moderate or reply to it centrally; the only correction that reaches a closed group is one a trusted member forwards in. That is why shareable, forwardable, truth-sandwich counter-content — designed to be passed hand to hand — is not a nice-to-have but the core format for this channel. [2,4]

Tool: the rapid-response protocol

Keep this on the wall. Speed and discipline are the whole game — but so is restraint: not every myth deserves the oxygen of a response.

1. **Detect.** One named person each week scans X, TikTok and WhatsApp for emerging SRHR claims (the "listening rota").
2. **Classify.** Which of the four typologies? Which channel? How fast is it moving, and is it crossing communities?
3. **Decide.** Ignore (low reach — do not amplify), monitor, or respond. Amplifying a fringe myth to "correct" it can spread it further.
4. **Respond.** Same channel, trusted voice, truth-sandwich structure, shareable format — within hours, not weeks.
5. **Escalate.** If it crosses your reach threshold, alert coalition partners and pre-identified faith and clinical allies. Pre-agree this threshold now, not during a crisis.

ENTRY POINTS FOR YOUR ORGANISATION • **Reaching the Achieng family group**

→ **Recruit the admins.** Every family group has a Beth. Train trusted group admins to recognise the four tell-tale signs and to post a calm correction in their own group — the only voice a closed group will accept.

→ **Seed forwardable counters.** Supply ready-made, truth-sandwich voice notes and images built to be forwarded into groups like this one, in Kiswahili and Sheng.

→ **Prebunk before the wave.** Teach the "miracle testimony / borrowed authority / urgent instruction / identity hook" pattern to youth and CHPs so the next Silas forward is met with recognition, not belief.

FOR YOUR PRACTICE • **Standing up your counter-disinformation capacity**

→ Assign a weekly listening rota and a single named crisis contact today — before the next wave.

→ Build a living "rapid-response bank" of truth-sandwich counter-messages, organised by typology, ready to localise.

→ Prebunk proactively: teach your audiences the four tell-tale signs of health disinformation so they spot the next one themselves.

→ Build faith-leader and clinician alliances in calm periods; they are your only fast lever in a crisis.

MODULE 3

03 Culture, Patriarchy & the Architecture of Restraint

The question this module answers: *If we build the clinic and subsidise the method, why don't they come?*

IN THIS MODULE

- Structural power, not individual attitudes: what the evidence really shows
- The gender continuum — from blame to transformation
- Dignity as an access condition: why stigma outranks logistics
- The six-part architecture of restraint
- The legal landscape every SRHR communicator should know
- What actually shifts men — and the myth that they are immovable
- Tool: costing a woman's decision, and a gender-transformative design check

THE STORY · MODULE 3 • **What it costs Mama Grace to decide**

Mama Grace has four children and does not want a fifth yet — she is exhausted, and school fees are already impossible. She has not discussed it with Baba Tom, because she knows how that conversation ends. So she walks to a chemist two estates away, where no one knows her, and hides her pills inside a rice tin at the back of the shelf.

One afternoon Tom finds them. To him it is not a health decision — it is a challenge to his authority and a sign of unfaithfulness.

Grace stops the pills for a year. Note what did not fail here: she knew the facts, a clinic existed, the method was affordable. What defeated her was not a knowledge gap or a logistics gap. It was power — the cost her decision carried once it was visible. That cost is the barrier this module is about.

ILLUSTRATION *A hand reaching into a rice tin at the back of a shelf, pill packet half-hidden — then the same tin open on a table with Tom's shadow across it. Secrecy and its discovery. Grace's face: not guilt, but fear of the cost.*

NARRATIVE STREAMS **S4 Patriarchy & Male Authority · S7 Policy & gatekeeping · S2 Moral & religious**

The evidence: a structural system, not a bag of bad attitudes

The most common and most costly mistake in SRHR programming is to treat patriarchy as a collection of individual bad attitudes to be corrected one man at a time. The Phase 1 research is unambiguous: across all four focus-group sites, men held disproportionate authority over whether contraception is used, how many children a couple has, when services are sought, and whether SRHR can be discussed at home at all. The decisive point is not what any individual man believes but what happens when a woman steps outside the accepted boundaries — the system's enforcement, not its opinions.[2,5]

The clearest fingerprint of that enforcement is secrecy. Roughly half of Kenyan women who use contraception do so without their partner's knowledge, and national research puts covert use among married women at around **12%** — highest among women with least education (22%) and the poorest households, and concentrated in concealable methods (injectables, implants). Covert use is not a quirk of individual couples; it is a population-level signal of unequal reproductive power.[5,16]

> *"My husband found my pills and threw them away. He said I was planning to cheat."*
> — Female, 24 · Nairobi informal settlements focus group[5]

The framework: from blame to transformation

FRAMEWORK • **The Gender Integration Continuum (IGWG / USAID)**

The standard tool for locating an intervention on the path from individual-blame to structural change places programmes on a continuum: **gender-blind** (ignores gender roles) → **gender-exploitative** (reinforces inequality to hit targets — discouraged) → **gender-accommodating / sensitive** (works around inequality; short-term gains, underlying system intact) → **gender-transformative** (actively transforms unequal relations and the structures that hold them).

Most SRHR communication sits, unexamined, at "accommodating" — it delivers a woman information while leaving the power that constrains her untouched. The evidence argues for moving deliberately toward the transformative end: engaging men as partners, dignifying services, and naming the structural drivers, not just the individual behaviour.[17,18]

Dignity is an access condition

If the barriers were mainly logistical, more clinics and cheaper methods would close the gap. They do not — because the barriers that bind hardest are social. In the national poll, the top two barriers by a wide margin were fear of being judged by the community (76%) and stigma attached to seeking services (72%), both ranking above availability, provider attitudes, cost and transport. Even women who overcome the fear are then stopped by a lack of privacy at the facility. Dignity — being able to seek care without judgment, surveillance or exposure — is therefore not a "soft" add-on. It is a precondition of access.[3]

Barrier to SRHR access	Share citing it	Category
Fear of being judged by community members	76%	Social
Stigma associated with seeking SRHR services	72%	Social
Limited availability of services at facilities	66%	Institutional
Health providers' attitudes to young/unmarried clients	65%	Institutional
Pressure from parents or partners	64%	Family
Cost of services or contraceptives	63%	Financial
Transport costs to facilities	62%	Financial
Cultural norms discouraging contraceptive use	60%	Social
Lack of privacy / confidentiality at facilities	58%	Institutional
Family disapproval of contraception / SRHR	58%	Family
Requirement for partner or parental permission	57%	Institutional/legal

Barriers to access (n=2,013): TIFA opinion poll[3].

Independent Kenyan research reaches the same conclusion. In a two-county study of adolescent services, social and attitudinal barriers — community stigma, hostile providers, breaches of confidentiality — were emphasised *more consistently* than logistical ones such as cost or distance. Being seen at a clinic marks an adolescent as sexually active; a single humiliating visit deters years of return. [19]

The architecture of restraint: six barrier domains

The research organises these binding barriers into six domains — a checklist for finding where your own programme leaks. Note that four of the six are about dignity, legitimacy and power; only two are the logistical barriers programmes usually target first. [2]

Domain	How it stops women
Provider judgment	Unmarried women are delayed, criticised, and questioned about the legitimacy of their need. One provider asked a client: "Why are you using family planning if you have no family?"
Geographic distance	Cost and distance are severe in Narok and rural Homa Bay, compounded by low awareness of what services exist.
Moral scrutiny	Fear of religious and community criticism turns the clinic into a site of surveillance; women seek services in secret.
Legal gatekeeping	Partner and parental consent requirements functionally deny services to unmarried women and adolescents.
Supply-chain failure	Stock-outs of ARVs, HIV test kits and state-supplied condoms reached crisis level during the study — a 13% reach decline — signalling state withdrawal.
Stigma by marital status	Unmarried women are treated as illegitimate users of SRHR services; marriage is framed as a prerequisite for care.

Barrier domains: ImaarishaSRHR research report [2].

The legal landscape every communicator should know

Communication does not happen in a legal vacuum, and getting the law wrong hands ammunition to the opposition. Article 43 of the Constitution guarantees every person the right to the highest attainable standard of health, including reproductive health care. Article 26(4) permits abortion where, in a trained professional's opinion, there is need for emergency treatment or the life or health of the mother is in danger, or as permitted by other written law — a broad "health" exception that is widely under-implemented. [20]

> FRAMEWORK · **Three legal turning points to cite accurately**
>
> **2018 — the Marie Stopes directive.** The medical board ordered a halt to abortion-related services after an advertising complaint; later challenged and lifted.
>
> **2019 — FIDA-Kenya v. Attorney General.** The High Court reinstated the withdrawn Standards and Guidelines and affirmed the constitutional right to abortion where physical, mental or social health is at risk, including in cases of rape.
>
> **2022 — PAK & Salim Mohammed v. Attorney General (Malindi).** The court held that abortion care is a constitutional right and that arbitrary arrest of patients and providers is unlawful. [21,22]

What actually shifts men

The evidence flatly contradicts the fatalistic view that men are an immovable obstacle. The leading reason men resist contraception is not deep ideology but misinformation and lack of accurate knowledge (45%) — a correctable, content-shaped problem. When Kenyans were asked what messages would move men, the top answers were shared decision-making between partners (53%), family wellbeing and financial stability (48%), and health benefits for women and children (47%). Human-rights framing came last (18%). The most trusted messenger to reach men is a health worker (63%).[3]

> CASE IN POINT • **Gender champions in Kisii and Kilifi — men as partners, not targets**
>
> The AQCESS project ran quarterly male–female community dialogues, trained community health volunteers on gender equality, and worked through respected "gender champions." Its endline evaluation documented partners who had previously prohibited contraception beginning to collaborate, joint decisions on child-spacing, and large increases in men accompanying women to antenatal care and facility delivery. The facilitators were specific: framing reproductive health as a shared responsibility, working through local respected leaders, and mentoring volunteers to reach men where they are.
>
> Crucially, what shifted resistant men was not a rights argument but a shared-benefit, family-welfare frame — "it has brought happiness in homes." This is the empirical basis for economic and family framing when engaging men (Module 5).[23]

Why men resist — and why most of it is fixable

It is worth dwelling on the composition of male resistance, because it determines the intervention. The largest single driver is not conviction but information failure — misinformation and lack of accurate knowledge (45%) — followed by concern about side effects (39%) and masculinity beliefs (35%). Only a minority cite fixed religious conviction (26%). The practical reading: most male resistance has a content-and-conversation solution, not an ideological one.[3]

Reason men resist contraception	Share	Nature of the fix
Misinformation / lack of accurate knowledge	45%	Correctable — accurate information, trusted messenger
Concerns about side effects / health impacts	39%	Correctable — method counselling, provider access
Masculinity beliefs (many children prove manhood)	35%	Norm change — role models, reframing provision
Cultural beliefs or traditions	34%	Norm change — elders, community dialogue
Desire to control reproductive decisions	31%	Structural — shared-decision framing
Religious beliefs or teachings	26%	Engagement — faith allies (Module 6)

Reasons men resist: TIFA opinion poll[3].

The messengers matter as much as the message: asked who would move men, Kenyans named health workers first (63%), well ahead of community elders (38%), media personalities (32%) and religious leaders (31%). A clinician saying "here is what actually happens to your wife's health, and your family's future" outperforms almost any other voice — and almost any rights argument.[3]

Tool: cost her decision, then lower the cost

This diagnostic converts the structural lens into design. For a real SRHR decision a woman in your community faces, work three columns.

1. **Her decision.** Name it concretely: start an injectable, visit the clinic alone, decline another pregnancy.
2. **What it costs her.** Socially, physically, economically — surveillance, a partner's reaction, a provider's judgment, lost privacy.
3. **What lowers the cost.** A discreet channel, privacy at the facility, a supportive-partner script, a trusted escort, a provider trained to welcome unmarried clients.

The gender-transformative design check

Before finalising any activity, ask: does this merely inform a woman (accommodating), or does it also reduce the power or the cost that constrains her (transformative)? If every line targets the individual and none touches the system, the intervention will accommodate the barrier rather than move it.

ENTRY POINTS FOR YOUR ORGANISATION • **Lowering the cost of Grace's decision**

→ **Design for discretion.** Offer methods and channels that reduce visibility (self-injectable DMPA-SC, discreet resupply), because for many women visibility is the barrier, not access.

→ **Fix your own front desk.** Audit your service points for the two barriers women rank highest — provider attitude and privacy — before spending on more outreach.

→ **Engage men as partners, not obstacles.** Reach the Toms with shared-decision and provision framing carried by a male health worker or role model (Module 5), so Grace's decision need not be secret.

FOR YOUR PRACTICE • **Designing for structure and dignity**

→ Add "what does this cost her to act on?" to every activity design, and track cost-reducing wins (privacy added, provider retrained), not only people reached.

→ Audit your own service points for the two barriers women rank highest — provider attitude and privacy — before investing in more reach.

→ Engage men with shared-decision and family-wellbeing framing carried by a health worker; lead with rights only after the door is open.

→ Get the law right in every message; cite Article 43 and the 2019/2022 rulings accurately so the opposition cannot exploit an error.

MODULE 4 · THE NEW FRONTIER

04 The Mother & Aunty Frontier

The question this module answers: *How do we counter a warning that arrives as love?*

IN THIS MODULE

- Why intergenerational transmission is Phase 2's highest-leverage point
- Social-norms theory: descriptive and injunctive norms
- Why trusted insiders — not outside campaigns — change norms
- The data on older women's influence
- From gatekeeper to champion: the reframing that works
- A proven model: engaging grandmothers as agents of change
- Tools: rewriting the warning and recruiting champion elders

THE STORY · MODULE 4 • **Beth and Agneta, in the kitchen**

It is early evening. Agneta is helping her Aunty Beth prepare the evening meal — peeling, stirring, the radio low in the background. This is where the family's real conversations happen, not in any clinic. Agneta finally says what has been weighing on her: she is terrified. She wants to be a nurse, she has seen girls in her class disappear after a pregnancy, and she does not know how to keep herself safe.

Beth stops stirring, takes her hand, and speaks with total love and total certainty.

Agneta nods. She trusts Beth more than any teacher, any nurse, any poster. The warning has landed — delivered as love, received as truth. And Beth is not wrong to care. She is only wrong about the facts inside her care.

ILLUSTRATION *The kitchen at dusk: Beth at the sufuria, Agneta beside her with a knife and a bowl, steam rising, warm lamplight. Two faces close together — this is intimacy, not confrontation. The same kitchen recurs whenever Beth speaks.*

NARRATIVE STREAMS **S5 Generational & Social Control · S6 Disinformation · S4 Patriarchy**

The evidence: the most trusted channel carries the most misinformation

This is the insight the sector has most under-used, and the research names it as Phase 2's highest-leverage intervention point. Mothers, aunties and mothers-in-law transmit SRHR beliefs — true and false — through the most intimate and trusted relationships in a young woman's life. Because the message arrives as love, protection and guidance, it bypasses the critical filtering that a stranger's claim would meet. That intimacy is exactly what makes harmful norms so durable, and conventional campaigns so weak against them.[2]

67%	64%	60%
say older women shape younger women's confidence to decide	say older women encourage adherence to traditional norms	say they influence seeking services openly vs secretly

Older-women influence (n=2,013): TIFA opinion poll[3].

The research calls this the **"Mother and Aunty Syndrome"**: patriarchal and cultural norms transmitted not only through men, but through older women themselves, who carry messages about fertility, obedience, purity and shame. The challenge is not malice. These women are *sincere* — passing on misinformation they received as truth, out of genuine care. That is precisely why a fact-check from a stranger cannot outweigh a warning from your own mother.[2,5]

> *"They say contraception causes infertility. Young women begin to feel their bodies are not theirs to control."*
> — Female, 18–35 · Nairobi informal settlements focus group[5]

The framework: why norms, not facts, are the battleground

FRAMEWORK · **Descriptive vs injunctive norms — why Agneta won't act on facts alone (S5)**

Behaviour is governed less by what people know than by two kinds of social norm: **descriptive norms** (what I believe others actually do) and **injunctive norms** (what I believe others approve of). Agneta may learn at school that contraception is safe (accurate belief) yet still avoid it, because she believes the woman who matters most — Beth — disapproves (injunctive norm), and that "good" girls in the Achieng family wait for marriage (descriptive norm). Her reference group, not the clinic, is deciding.

This maps directly onto Narrative Stream S5 (Generational & Social Control), which the social-listening analysis found carries shame-based language and intergenerational norm enforcement. The implication is decisive: handing Agneta more facts does not move the norm. What moves it is changing what she believes *Beth* does and approves — which means working through Beth, not around her. Older women are not a wall to go around; they are the reference group itself.[24,25]

FRAMEWORK · **Diffusion of innovations — change travels through trusted insiders**

New norms spread through social systems along lines of trust, carried by respected "opinion leaders" whose adoption gives others permission to follow. Outside authorities can supply information, but insiders supply legitimacy. This is why a respected older woman who visibly changes her stance can shift a family or a village in a way no external campaign can: she is the credible early adopter her network already watches.[26]

The reach of older women's influence

The national data show older women's influence is broad and specific — it touches confidence, norms, and the crucial question of secrecy.[3]

Older women's influence on younger women...	Influential
Shaping confidence to make independent SRHR decisions	67%
Encouraging adherence to traditional norms on sexuality/childbearing	64%
Reinforcing "men should decide on family matters"	64%
Reinforcing "a good wife must give birth soon after marriage"	60%
Whether young women seek services openly or secretly	60%

Older women's influence on younger women...	Influential
Promoting fear of family shame over suspected contraceptive use	60%
Prioritising marriage and childbearing over education or work	53%

Older-women influence: TIFA opinion poll[3].

The transmission runs in a clear chain: **older women** → carry **messages** (contraception myths, fertility pressure, shame, early marriage, obedience) → shaping **young women** (constrained autonomy, delayed care-seeking). The good news hidden in the same finding: the channel is not fixed. If it can carry a harmful message, it can carry an accurate one.[2]

From gatekeeper to champion

The strategy is not to bypass or discredit older women — attacking a young woman's mother will fail, and deservedly. It is to reframe the older woman's role from gatekeeper of a harmful norm to champion of her daughter's health. This works because it honours her intention (protecting the young woman) while updating her information. She is not told she was wrong to care; she is given a more accurate way to express the same love.[2]

The old script (received as love)	The new script (same love, accurate information)
"Family planning will make you barren — I'm protecting your future children."	"I want you to have healthy children when you're ready — spacing births is how strong mothers plan their families."
"Only loose women go to the clinic alone."	"A woman who looks after her health is a responsible woman — I'll come with you if you like."
"Using contraception means you plan to be unfaithful."	"Planning your family together is what respectful partners do — it protects the whole home."
"We don't talk about these things in this family."	"No one told me these things — but you can ask me, and we'll find the right answer together."

There is direct evidence this is possible. The research notes that some older women in urban settings are already beginning to encourage education and contraception, and that a faith-and-culture channel can be redirected toward protection — the **Maasai elders' anti-FGM declarations** captured in the social-listening data show the very authority that once enforced a harmful practice being turned against it. Orientation shifts — with deliberate, specific engagement.[2,4]

> CASE IN POINT · **A proven model: engaging grandmothers as agents of change**
>
> The most rigorously documented version of this strategy is the Grandmother Project's "Change through Culture" approach, developed in West Africa. Rather than treating grandmothers as obstacles to bypass, it positions them as respected, experienced advisors and convenes intergenerational dialogue in which grandmothers themselves reconsider and update norms on child marriage, teenage pregnancy, nutrition and girls' education. Reviews of the evidence conclude that grandmothers are a near-universal, influential and consistently neglected resource in family and child health, and that engaging them as allies — not targets — shifts deeply held norms where information campaigns alone stall.
>
> The transferable principles for Kenya's Mother & Aunty frontier are exact: engage older women with respect for their role, create intergenerational dialogue rather than one-way messaging, and let respected elders model the new norm for their peers.[27,28]

Reaching older women where they actually are

Design for the right channel. Older women are Level 1 (intimate-circle) messengers whose infrastructure is WhatsApp and community radio — not TikTok. Produce forwardable WhatsApp voice notes and images in Kiswahili and local languages, run community-radio conversations, and recruit champion older women who model the new scripts in real family settings. The Community Health Promoter network is the natural ally: the same trusted, doorstep relationship that can carry a mother's warning can be briefed to carry accurate scripts instead. [2,3]

Designing an intergenerational dialogue

The grandmother evidence points to a specific method: not one-way messaging at older women, but structured dialogue that lets them reconsider norms among their peers and with younger women present. The design principles are consistent across the successful models. [2]

- **Convene, don't lecture.** Bring older and younger women together with a trusted facilitator; the goal is reflection, not instruction.
- **Start from shared values.** Open on what everyone wants — healthy daughters, strong families — before any contested fact.
- **Honour experience.** Invite older women to speak first, as advisors; their standing is the asset, not the obstacle.
- **Let elders reach the new norm themselves.** A conclusion an older woman arrives at, she will defend to her peers; one imposed on her, she will resist.
- **Close on a concrete commitment.** "What is one thing we will do differently for our daughters?" — named aloud, in the group.

What not to do

Three moves reliably backfire on this frontier: mocking or caricaturing older women (it severs the relationship you need); framing them publicly as the problem (it makes them defend the old norm harder); and bypassing them to reach the young woman directly (it pushes her decision underground into secrecy, the very pattern the data show). The older woman is the channel — work with her, never around her. [2,5]

Tool: rewrite the warning

A repeatable method for converting a harmful-but-loving warning into an accurate one that keeps its emotional authority intact.

1. **Collect the real warning.** Use the actual words older women in your community use — not a paraphrase.
2. **Find the loving intention underneath.** Protection, honour, family stability — name it explicitly.
3. **Rewrite to keep the intention and the intimacy, change the facts.** The sentence should still sound like a mother, not a poster.
4. **Test it aloud with real older women.** If it sounds like a campaign slogan, it will not travel. Keep the love in the sentence.

Recruiting champion elders — selection criteria

Not every older woman is a champion. Look for: standing and respect in her network (an opinion leader others watch); a personal openness to updating her views; comfort speaking in family and community settings; and — ideally — her own change-of-heart story, which is the most persuasive content of all.

> ENTRY POINTS FOR YOUR ORGANISATION • **Reaching the Beths in your community**
>
> → **Recruit "champion aunties."** Identify and train respected older women like Beth — WhatsApp group

admins, women who run family kitchens — to model the new scripts. This is the single highest-leverage programme this guide points to.

→ **Get into the family WhatsApp groups.** Produce forwardable Kiswahili voice notes and images designed to travel in groups like "Achieng Family" — the channel Beth already runs.

→ **Brief your CHPs for the kitchen.** Equip community health promoters with the old-script/new-script method for exactly the doorstep moment Beth and Agneta are having.

→ **Run intergenerational dialogues.** Convene Beths and Agnetas together with a facilitator — see the standalone Mother & Aunty curriculum for full session plans.

FOR YOUR PRACTICE · **Working the intergenerational channel**

→ Recruit and support "champion" older women who model the new scripts in real family settings, and give them their own change-of-heart stories to tell.

→ Produce Mother & Aunty content for WhatsApp and community radio — not youth platforms — in Kiswahili and local languages.

→ Brief CHPs and mobilisers on the old-script/new-script method so they reinforce it on home visits.

→ Never frame older women as the enemy in any message — frame them as protectors carrying new information.

MODULE 5

05 Reframing SRHR

The question this module answers: *If our message is true, why isn't it landing?*

IN THIS MODULE

- The definition gap: where the audience actually starts
- Framing theory — why the frame beats the fact
- "Misnomerization": how naming narrows meaning
- The messages that shift acceptance — and the ones that don't
- The five-part reframing formula, with worked examples
- The messenger architecture and the power of language
- Tools: the reframing worksheet and the 30-second framing check

THE STORY · MODULE 5 • **The poster that failed, and the word that worked**

The NGO poster on the estate wall said: "Your body, your right to choose." It was true, and it changed nothing in the Achieng home — worse, it hardened Baba Tom, who read "rights" as the outside world unsettling his household.

Then Nurse Wanjiru visited during a health drive and spoke to Tom differently. She did not mention rights at all.

Tom listened. Same underlying fact — spacing births — but arriving through a value he already held (providing for and protecting his family) instead of a value he experienced as a threat. Wanjiru had not watered anything down. She had simply chosen a door Tom would walk through. That is reframing.

NARRATIVE STREAMS S1 Health & rights · S3 Cultural identity · S4 Patriarchy

The evidence: the audience is not where the sector is

Reframing is the practical heart of this guide, and the evidence for it is stark. When 2,013 Kenyans were asked what comes to mind on hearing "sexual and reproductive health," 30% said healthcare or clinics, 22% contraception, 18% pregnancy — and only 8% named women's rights, 2% wellbeing or relationships. The sector lives in rights language; the public starts at health and family. A campaign built on rights is speaking a language most of its audience has not adopted.[3]

"First thing that comes to mind" with SRHR	Total	Male	Female
Healthcare / infant health	30%	31%	30%
Contraception	22%	18%	26%
Morality	9%	11%	7%
Abortion	8%	7%	8%
Women's rights / bodily control	8%	8%	8%

"First thing that comes to mind" with SRHR	Total	Male	Female
Youth behaviour	7%	8%	6%
Wellbeing / relationships / self	2%	1%	2%

Associations with SRHR (n=2,013): TIFA opinion poll[3].

The framework: why the frame beats the fact

FRAMEWORK · **Framing theory — the first words decide how the rest is heard**

People do not process information neutrally; they interpret it through "frames" — mental structures triggered by the first cues in a message. The same fact framed as a right, a health benefit, or a family duty activates different values and reaches different people. Communication research (notably the FrameWorks Institute's work on strategic framing) shows two rules that matter here: lead with values and solutions rather than crisis and blame; and never repeat your opponent's frame to negate it, because repeating a frame strengthens it. "Contraception is not about promiscuity" reinforces the link between contraception and promiscuity in the listener's mind. Say what it is about — healthy families — not what it is not.[29,30]

FRAMEWORK · **"Misnomerization" — how a name narrows a right**

The Phase 1 desk research identifies a specific distortion mechanism it calls **misnomerization**: the way everyday naming quietly shrinks SRHR from a framework of rights, dignity and autonomy down to a narrow women's clinical service. In the focus groups, SRHR was named *"clinic ya wamama"* (the women's clinic) and *"afya ya wamama"* (women's health), and contraception was softened to *"planning for children."* Each name feminises SRHR, narrows it to a clinic visit, and erases consent, autonomy and men's stake in it.

The lesson is not to impose clinical or rights jargon, but to choose everyday language that opens the frame rather than closing it — language of health, family, planning and responsibility that still carries dignity and choice inside it.[2,5]

The messages that shift acceptance

Asked which messages would most influence their community, Kenyans chose health benefits for women and families (47%), family wellbeing and responsible parenting (45%), and the economic benefits of planning (37%). Human rights and bodily autonomy came last, at 18%. This is not an argument against rights — it is an argument about sequence. Health is the entry point; family wellbeing is the door; rights is the destination.[3]

Message	Most influential (%)
Health benefits for women and families	47%
Family wellbeing and responsible parenting	45%
Economic benefits of planning / smaller family size	37%
Protecting young people's future and opportunities	22%
Faith-based perspectives that support family health	19%
Human rights and bodily autonomy	18%

Most influential messages: TIFA opinion poll[3]. Reframing is sequencing, not dilution.

The public even tells you what reframing requires. Asked what would make SRHR seen as a health right rather than a moral issue, the top answers were more education and awareness (59%), endorsement from community and religious leaders (46%), and better public messaging that frames SRHR as a health issue (45%). The mandate for values-based, leader-endorsed, health-framed communication is explicit in the data.[3]

The five-part reframing formula

Use this structure to rebuild any SRHR message. Each step is anchored in the evidence from Modules 2–4.

1. **Open on a shared value.** Health, a strong family, a responsible decision, a mother's wellbeing — never rights language first.
2. **Name any myth once, without amplifying it.** Truth-sandwich structure (Module 2): fact first, myth once, fallacy, fact.
3. **Deliver the fact through a trusted messenger.** A clinician (71% trust), a survivor, a peer, a champion older woman — trust, not title.
4. **Make it shareable and local.** Kiswahili or Sheng; WhatsApp-forwardable; one clear idea per message.
5. **Point to dignified action.** Where to get safe, accurate information or services — without judgment.

Reframing in action

Rights-first (low reach)	Values-framed (meets the audience)
"Contraception is your reproductive right."	"Spacing your children is how a family stays healthy and strong — and it's your choice to make."
"CSE protects children's bodily autonomy."	"When young people get accurate answers early, they make safer, more responsible decisions."
"Abortion access is a human right."	"Every woman deserves safe, accurate, non-judgmental care — whatever her situation."
"Men must stop controlling women's bodies."	"When partners decide together, the whole family is stronger — shared decisions, shared future."

The messenger architecture: who carries which message

Trust is not uniform; it varies by messenger, message and moment. Healthcare workers are trusted most (71%), yet only about a third of women had a substantive SRHR conversation with a provider in the past year — the most credible voices are largely silent. The research maps three trust levels, and the strategic art is matching each message to the right level.[3,2]

Level	Messengers	Best used for	Primary channels
1 · Intimate circle	Mothers, aunties, partners	Reframing warnings; normalising open family conversation	WhatsApp, community radio
2 · Institutional	Healthcare workers, CHPs, teachers	Authoritative myth-busting; method facts	YouTube, website, podcast
3 · Public sphere	Faith leaders, influencers, media	Wide reach; youth reach via TikTok	TikTok, Instagram, X,

Level	Messengers	Best used for	Primary channels
			Facebook

Messenger architecture: ImaarishaSRHR research report [2].

CASE IN POINT • **Trusted messengers beat message volume**

The reach data make the case for messenger choice unmistakable. In the social-listening dataset, the faith account @Prophet_DrOwuor averaged 540,271 people per post — roughly seven times the per-post reach of @NationAfrica (76,437) — despite posting far less often. When a faith actor and a media institution posted about SRHR on the same day, the faith actor reached a larger and more emotionally engaged audience every time.

The strategic conclusion is not to out-shout the loudest account. It is to invest in the few messengers your audience already trusts — a named clinician, a survivor advocate, a champion elder, a respected faith ally — and let their credibility, not your posting volume, carry the message. [4]

Language is a belonging signal

Which language a message uses is itself a message. In the social-listening data, Sheng-language content outperformed English by roughly 3:1 in sharing among 18–24-year-olds; the strongest Kenyan youth-media brands built their credibility on authentic Sheng and characters, not translated English campaigns. Language signals belonging, and belonging drives sharing. English-first is, for youth audiences, a reach problem. [4,31]

Framing for specific audiences

The same accurate content needs a different entry frame for each audience, because each holds different values most closely. This is the reframing formula applied audience by audience — a ready reference for message design. [3,2]

Audience	Lead with...	Avoid leading with...	Trusted messenger
Men / partners	Shared decisions; providing for and protecting the family	"Your control over her body"	Health worker; male role model
Older women	Protecting daughters' health; strong, planned families	"Your beliefs are wrong / outdated"	Champion elder; CHP
Youth 18–24	Confidence, accurate answers, no judgment	Clinical or moralising tone	Peer creator (Sheng); clinician
Faith communities	Family wellbeing; care for the vulnerable	Rights language that reads as anti-faith	Supportive faith leader
General public	Health benefits for women and families	"Human rights and bodily autonomy" first	Doctor / nurse

Audience framing synthesised from TIFA (messages and messengers) and the research report [3,2].

Notice that rights framing is never absent — it is the destination for every audience. What changes is the door. For the faith community the door is care for the vulnerable; for men it is provision and partnership; for youth it is confidence and honest answers. Arriving at rights through the right door is the whole craft of reframing. [2]

A full rebuild, start to finish

Here is the whole method applied to one real message an SRHR organisation might publish — a clinic poster on contraception — showing every step of the transformation.[2]

Before (rights-first, low reach)	After (values-framed, evidence-led)
Headline: “Contraception is your right. Don’t let anyone deny it.”	Headline: “Plan your family. Protect your health.”
Body: “Every woman has the right to control her own body and reproductive choices.”	Body: “When you space your children, you and your babies stay healthier — and it’s your decision to make, together with the people you trust.”
Myth handling: none — or “Family planning does NOT make you barren.”	Myth handling (truth sandwich): “Your body returns to normal after you stop — your fertility comes back. A health worker can show you how each method works.”
Messenger: the organisation’s logo.	Messenger: a named local nurse, pictured — “Ask Sister ____ at ____ clinic.”
Channel & language: English poster at the facility.	Channel & language: Kiswahili; a forwardable WhatsApp image and a community-radio spot as well as the poster.

The facts did not change — fertility returns, the choice is hers, the method is safe. What changed is the door: health and family instead of rights, a trusted nurse instead of a logo, Kiswahili on WhatsApp instead of English on a wall, and the myth defused rather than ignored. That is reframing — and it is the difference between a message the audience scrolls past and one they act on and forward.[3,2]

Tool: the reframing worksheet

Take one real message your organisation has published and run it through five checks, then rebuild it with the formula.

1. **Does it open on a shared value or on rights?** If rights-first, re-sequence.
2. **Does it repeat the opposition’s frame?** If it says what SRHR is “not,” rewrite to say what it is.
3. **Is the messenger trusted by this audience?** Match to Level 1/2/3.
4. **Is it in the right language and forwardable?** Kiswahili/Sheng, one idea, shareable.
5. **Does it end on dignified action?** A concrete, non-judgmental next step.

The 30-second framing check (print and pin)

Before any SRHR message goes out, does it: open on health or family, not rights? come from or quote a trusted messenger? name any myth once and lead with the correction? work in the right language and channel for its audience? and end by pointing to safe, accurate information or dignified services? No message skips the check.

> ENTRY POINTS FOR YOUR ORGANISATION · **Finding the right door for each person**
>
> → **Write to the person, not the population.** Before drafting, name who you are reaching — a Tom, a Beth, an Agneta — and the one value they hold most closely; open there.

→ **Retire your rights-first assets.** Audit existing posters and posts; the ones that open on “rights” are your lowest-reach material. Rebuild them with the five-part formula.

→ **Put the fact in a trusted mouth.** Voice health facts through a Nurse Wanjiru (Level 2) and norm-shifts through a champion elder or peer (Level 1/3) — match message to messenger before publishing.

FOR YOUR PRACTICE · **Making values framing your default**

→ Adopt the five-part formula as your standard message template, and add the 30-second framing check to content approval.

→ Match every core message to a trusted messenger before publishing — invest in a few trusted voices, not sheer volume.

→ Build youth content in Sheng and on the platforms youth use; never let English-first be the default.

→ Say what SRHR is, never only what it is not — and choose everyday names that open the frame, not narrow it.

MODULE 6

06 What Works

The question this module answers: *What has actually moved SRHR outcomes in Kenya — and what can we build on?*

IN THIS MODULE

- Proof points, not pilots: the Kenyan evidence of what works
- Why layered, multi-channel models win
- Trusted human intermediaries: CHPs, faith allies, peers
- The digital landscape — reach, limits and the gender gap
- Community radio: the most cost-effective channel
- An honest look: where well-funded programmes fell short
- Tool: the channel-and-messenger selection matrix

THE STORY · MODULE 6 • **The corner that changed Agneta's mind**

A year on, four things reached Agneta — none of them a poster. At the health centre, Nurse Wanjiru ran a youth-friendly corner where no one asked why an unmarried girl had come. On TikTok, her cousin Kevo posted a ninety-second Sheng clip busting the "Depo makes you barren" myth that got more shares than anything the county had produced. At church, Rev. Otieno — after a quiet conversation with a CHP — began framing planning as caring for the family God gave you. And Brenda, her friend who had left school pregnant, told her own story at a community forum, plainly and without shame.

Not one of these was new money or new technology. Each was a trusted person, in a trusted channel, carrying an accurate message. Together they did what a year of leaflets could not: they gave Agneta both the facts and the permission to act on them. This module is the evidence that these approaches work — and that Kenya already has them.

NARRATIVE STREAMS S1 Health & rights · S2 Faith as ally · S5 Youth digital · Bright spots

The evidence: proof points your strategy can stand on

The Kenyan record is not only a catalogue of barriers; it contains programmes that demonstrably moved outcomes. These are the proof points Phase 2 can build on rather than reinvent — and knowing them lets you argue from evidence, not hope. One cross-cutting lesson runs through all of them: layered, multi-channel models beat any single channel. Radio plus mobile plus interpersonal outperforms any one alone, because the forces they counter are themselves connected.[32]

Proven approach	What the evidence shows	Source
Community Health Promoters	A trusted doorstep channel at national scale (100,000+ CHPs); community-based provision of injectables by CHWs shown safe and feasible in Kenya.	Amref; Stanback et al. (GHSP)
Faith-based FP engagement	Religious leaders reached 675,000 people and referred 85,810 clients; facility FP visits rose up to sixfold.	CHAK / Ruark et al. (GHSP)
Urban multi-channel FP (Tupange)	Modern contraceptive use rose 45%→52%; long-acting methods 6%→19%; radio was	Winston et al.

Proven approach	What the evidence shows	Source
	the most cost-effective component.	(Guttmacher)
Survivor-led / peer testimony	Peer-led SRH education improves knowledge, testing and safer behaviour; narrative testimony changes health behaviour.	Peer-education review; narrative review
Clinicians under their own names	Doctors posting as themselves generate far more trust than institutional accounts; 71% trust health workers.	Social listening; TIFA poll
Sheng-language youth media	Sheng content outperforms English ~3:1 in sharing; Shujaaz reaches millions of youth.	Social listening; Kimani et al.

Full citations for each row appear in the Sources list and the cases below.

CASE IN POINT · **CHAK — turning faith facilities and leaders into an FP engine**

Between 2013 and 2017 the Christian Health Association of Kenya engaged faith-based facilities, religious leaders and community health volunteers in western Kenya. Religious leaders reached 675,000 men and women with family-planning information and referred 85,810 clients to facilities. Family-planning visits at phase-one facilities rose sixfold (705 to 4,286), with tenfold increases for pills, IUDs and implants; across all facilities, new implant visits more than tripled. The authors concluded the model is effective, scalable and sustainable.

This is the single strongest Kenyan proof that faith actors can move from gatekeepers to allies — the counterpart, on the constructive side, to the disinformation risk in Module 2. Faith engagement is not naïve; it is evidence-based.[33]

CASE IN POINT · **The Community Health Promoter programme — a trusted channel at national scale**

In 2023 Kenya professionalised roughly 100,000 community health volunteers into salaried, kitted Community Health Promoters — two per Community Health Unit — distributing 100,000 digital smartphones from September 2023 and anchoring the model in the Primary Health Care Act (2023). Amref reports over 107,000 CHPs nationwide, with rising immunisation coverage and earlier antenatal access in supported areas. Kenyan research had already shown that community-based provision of injectable contraception by community health workers is safe, feasible and well-accepted.

For SRHR communication, the CHP network is the infrastructure of the Level 1 channel — the same doorstep relationship that carries misinformation, available to carry accurate scripts. Briefing and partnering with CHPs is among the highest-leverage moves an SRHR organisation can make.[34,35]

The digital landscape: reach, and its limits

Digital channels are essential but not universal, and designing as if they were excludes exactly the communities that need most. Kenya has about 57 million people, 27.4 million internet users (48%), and roughly 15.1 million social-media user identities (about 26.5% of the population), skewed male (about 60% male, 40% female). TikTok is the fastest-growing platform. Two design consequences follow: reach youth on the platforms and in the language they use, but never rely on digital alone for rural women — pair it with radio and CHPs.[36]

48%	26.5%	60/40

internet penetration (27.4M users)	social-media penetration (~15.1M identities)	male-to-female skew in social-media use

Digital statistics: DataReportal "Digital 2025: Kenya" [37].

The research is emphatic that TikTok is *"Kenya's most influential youth SRHR learning space, and its biggest information risk"*. Young people arrive there with fears — pain, infertility, side effects — not abstract curiosity, and the comment sections are the real curriculum. Healthcare providers who use creator-style formats outperform institutional campaigns; content must be myth-responsive, emotionally honest, and available in English, Kiswahili and Sheng. [2]

Community radio: the cheapest channel that reaches the unreached

Radio is easy to overlook and hard to beat on cost-effectiveness. In the Tupange evaluation, radio was the single most cost-effective component. In a Nairobi informal settlement, a live SRHR radio programme not only raised awareness but opened previously taboo parent–child conversations and shifted gender-norm attitudes — and community radio reaches rural, low-literacy and linguistically diverse audiences that digital channels miss entirely. For Narok and Homa Bay, which are near-absent online but intense offline, radio is not a supplement; it is a primary channel. [38]

An honest look: where well-funded programmes fell short

> FRAMEWORK • **Design for realistic outcomes and long horizons**
>
> Not every well-resourced programme succeeds, and pretending otherwise weakens a strategy. A rigorous population cohort in Siaya County found that the large, multi-component DREAMS partnership did not, by itself, accelerate the decline in HIV incidence among adolescent girls and young women — although it did improve knowledge of HIV status and shift some gender-norm attitudes. The lesson is not that layered programmes fail, but that behaviour and norm change take time, and that programmes should be judged on realistic intermediate outcomes over long horizons, not on headline endpoints alone. [6]

Peer and survivor voices: the evidence for testimony

Two of the most powerful — and most under-resourced — Kenyan assets are peer educators and survivor advocates. The research literature is clear that peer-led SRH education improves knowledge, HIV testing and safer behaviour, and empowers the educators themselves, while narrative testimony changes health-promoting behaviour by building identification and self-efficacy in the listener. The Phase 1 data echo this: personal stories were among the online content types Kenyans said most influenced their SRHR decisions (34%), close behind medical information from professionals (45%). [39,40,3]

The practical caution the evidence attaches is safeguarding: survivor testimony must be voluntary, consented and supported, never extractive or re-traumatising. A survivor tells her story on her terms, with a duty of care around her — that is both an ethical requirement and what keeps the testimony credible. [41]

Tool: the channel-and-messenger selection matrix

For each audience, decide the primary channel and the trusted messenger before you design content. A worked starting point:

Audience	Primary channel	Trusted messenger	Framing lead
Youth 18–24 (urban)	TikTok, Instagram (Sheng)	Peer creators + clinicians	Confidence, accurate answers
Women 18–35	WhatsApp, radio, YouTube	Clinicians, survivors, peers	Health & family wellbeing
Older women	WhatsApp, community radio	Champion elders, CHPs	Protecting daughters' health
Men / partners	Radio, WhatsApp, barazas	Health workers, male role models	Shared decisions, provision
Rural (Narok, Homa Bay)	Community radio, CHPs, barazas	CHPs, elders, chiefs	Health, dignity, local language

Channel/messenger logic synthesised from TIFA, social listening and the research report [3,4,2].

Putting it together: an illustrative 90-day sequence

The pieces combine into a phased sequence. This is illustrative, not prescriptive — the point is that reach is built by layering trusted channels over time, not by launching everything at once. [2]

Phase	Focus	Moves
Days 1–30 · Set up	Infrastructure and trust	Stand up the listening rota; build the trusted-messenger map; recruit 2–3 clinicians and champion elders; agree the rapid-response protocol.
Days 31–60 · Seed	Content and relationships	Publish the first values-framed, clinician-voiced content on WhatsApp, radio and TikTok; brief CHPs on the new scripts; begin faith-leader conversations.
Days 61–90 · Amplify & respond	Reach and reaction	Run intergenerational dialogues; clip and distribute across channels; execute at least one live rapid response; review reach and reuse in the monthly review.

Sequence synthesised from the research report's strategic priorities [2].

ENTRY POINTS FOR YOUR ORGANISATION • **Assembling Agneta's corner in your area**

→ **Stand up a youth-friendly desk.** Partner with a Nurse Wanjiru; the fix is often training and privacy, not new buildings.

→ **Recruit a Kevo.** Fund one or two credible Sheng creators over glossy in-house content; language is a belonging signal.

→ **Turn a Rev. Otieno.** Invest in pre-crisis faith-leader relationships (the CHAK model) — your strongest ally and your fastest lever in a disinformation wave.

→ **Support the Brendas — safely.** Build a consented, safeguarded survivor-advocate network; peer testimony outperforms institutional messaging.

FOR YOUR PRACTICE • **Building on what works**

→ Amplify trusted, proven local approaches — CHPs, faith allies, survivor networks, Sheng creators — rather than importing generic formats.

→ Layer channels deliberately: radio plus WhatsApp plus interpersonal beats any single platform.

→ Never rely on digital alone for rural women; pair every digital push with radio and CHP reach.

→ Judge your work on realistic intermediate outcomes over time, and be honest with funders about horizons.

MODULE 7

07 Making It Routine

The question this module answers: *How do we keep all of this alive after the workshop energy fades?*

IN THIS MODULE

- The operating system: standing mechanisms, not one-off training
- The five habits and who owns them
- The strategic priorities this guide maps to
- 90-day signs it is working

From a guide to an operating system

A guide only changes an organisation if it changes routines. The research's own recommendations are institutional, not inspirational: it calls for a standing SRHR Narrative & Misinformation Observatory, a Trusted Messenger Network, and an always-on youth digital strategy — mechanisms that run every week, not a workshop that happens once. This short module turns the whole guide into a small set of habits your team can actually sustain. [2]

The five habits

1 · The listening rota (the Observatory in miniature). One named person each week scans X, TikTok and WhatsApp for emerging SRHR claims and flags them to the team — the front line of rapid response (Module 2).

2 · The living message library. Every good values-framed message and truth-sandwich counter is saved to one shared folder, tagged by topic and typology, ready to localise and reuse.

3 · The trusted-messenger map. A current list of your clinicians, survivors, champion elders, peer creators and faith allies — what each will speak on, and how to reach them fast.

4 · The 30-second framing check. No SRHR message goes out until one person has run the five framing questions (Module 5). No exceptions.

5 · The monthly 20-minute review. Once a month the team reviews what myths surfaced, what was published, what got shared, and what to reframe next.

Who owns what

Habit	Owner	Cadence
Listening rota	Rotating team member	Weekly
Message library upkeep	Comms lead	Ongoing
Trusted-messenger map	Community / partnerships lead	Monthly refresh
30-second framing check	Whoever publishes the content	Every message
Monthly review	Team lead facilitates	Monthly
Rapid-response escalation	Named crisis contact + coalition	As needed

The strategic priorities this guide serves

The Phase 1 research distilled its findings into a set of strategic priorities. Use this as a one-page map of where each module's practice fits the larger strategy.[2]

Strategic priority (from the research)	Where this guide equips you
Reframe SRHR from moral controversy to health, dignity and rights	Module 5 — values framing and the reframing formula
Build a sustained counter-disinformation & rapid-response system	Module 2 — typologies, protocol, prebunking
Prioritise trusted local messengers over message volume	Modules 5 & 6 — messenger architecture and proof
Engage men and boys as part of the solution	Module 3 — what shifts men; shared-decision framing
Work directly with mothers, aunties and older gatekeepers	Module 4 — the Mother & Aunty frontier
Structured religious and cultural leader engagement	Modules 2 & 6 — faith risk and faith as ally (CHAK)
Youth-centred digital strategy — TikTok and X	Modules 5 & 6 — Sheng, platforms, creators
Differentiated geographic strategy	Modules 2 & 6 — region-by-region targeting

Strategic priorities: ImaarishaSRHR research report (condensed)[2].

Common failure modes — and how to avoid them

Most SRHR communication efforts fail in predictable ways. Naming them in advance is the cheapest insurance.[2]

- **The one-off campaign.** A burst of activity around a day of observance, then silence — while disinformation runs always-on. Fix: standing habits, not campaigns.
- **Rights-first by reflex.** Leading with entitlement language the audience has not adopted. Fix: the framing check on every message.
- **Volume over trust.** Chasing posting frequency instead of investing in a few trusted messengers. Fix: the messenger map.
- **Digital-only.** Reaching Nairobi's feeds while Narok and Homa Bay hear nothing. Fix: radio and CHPs for the offline-intense counties.
- **Fighting the myth on the myth's terms.** Repeating and amplifying false claims to "correct" them. Fix: the truth sandwich and the decide-not-to-respond rule.
- **Treating older women as the enemy.** Bypassing or shaming the intergenerational channel. Fix: champion elders and dialogue.

90-day signs it is working

- Staff instinctively open SRHR messages on health and family, not rights — without being reminded.
- The message library and rapid-response bank are growing and actually being reused.
- Emerging myths are flagged within days, not discovered weeks later.
- Trusted clinicians, survivors and champion older women appear regularly in your content.
- Your team can name which "gear" of the system (Module 1) a new problem touches — and design across two at once.

> FOR YOUR PRACTICE · **Standing up the operating system**

→ Assign the five habits to named owners this week, and slot each into an existing meeting rather than creating new ones.

→ Agree your rapid-response escalation threshold and crisis contact before the next disinformation wave.

→ Set a 90-day review against the markers above, and adjust what is not sticking.

APPENDIX A

Myth & Fact Quick-Reference

A field card for CHPs, peer educators and mobilisers. Use the truth-sandwich rule: name the myth once, lead with the fact and a trusted source, and never make the false claim the headline.

The myth (name it once)	The fact (lead with this)
Family planning makes you barren.	Modern contraception does not cause infertility; fertility returns after stopping. (47% of Kenyans still believe otherwise — say the fact, not the fear.)
The pill / injection damages your womb.	These methods are safe and well studied; if one doesn't suit you, a provider can switch you to another.
Contraception means a woman plans to be unfaithful.	Family planning is how partners protect the whole family's health and plan their future together.
Abortion always destroys the womb / causes lifelong infertility.	Safe, legal care where the law allows does not cause infertility; unsafe procedures — driven by stigma and secrecy — are what harm women.
HIV can be cured by faith — stop your ARVs.	There is no faith or herbal cure for HIV; ARVs keep people healthy and alive. Never stop treatment. (See #ScienceBows, Module 2.)
CSE is foreign values that will corrupt our children.	Age-appropriate, accurate information helps young people make safer, more responsible decisions.
Only "loose women" or sex workers use the clinic.	Looking after your reproductive health is responsible and normal — for every woman, married or not.

Belief prevalence: TIFA opinion poll[3]; disinformation cases: ImaarishaSRHR social listening (see Sources).

APPENDIX B

Trusted-Messenger Map (template)

Keep this current — it is your fastest asset in a disinformation crisis and your first stop when planning any campaign. Fill a row for each real person, matched to the messenger levels in Module 5.

Messenger (name / role)	Trust level	Will speak on	Best channel	Contact
Dr. ____ (clinician)	Level 2 · high	Contraceptive safety, HIV facts	Podcast, YouTube, WhatsApp	____
Mama ____ (champion elder)	Level 1 · highest	Mother & Aunty scripts	WhatsApp, community radio	____
____ (survivor advocate)	Level 1–2	Personal testimony	Video, community forums	____
____ (Sheng creator)	Level 3 · youth	Myth-busting for 18–24	TikTok, Instagram	____
Rev / Sheikh ____ (faith ally)	Level 1 + 3	Pre-crisis alliance, HIV, FP	Pulpit, radio, WhatsApp	____
CHP ____ (community health promoter)	Level 1–2	Method facts, referrals	Home visits, WhatsApp	____

APPENDIX C

Rapid-Response Protocol (one page)

The five steps

1. **Detect.** Weekly listening rota scans X, TikTok and WhatsApp for emerging SRHR claims.
2. **Classify.** Which of the four typologies? Which channel? How fast, and crossing which communities?
3. **Decide.** Ignore (low reach — do not amplify), monitor, or respond.
4. **Respond.** Same channel, trusted voice, truth-sandwich structure, shareable format — within hours.
5. **Escalate.** Past your reach threshold, alert coalition partners and faith/clinical allies.

When NOT to respond

Restraint is part of the discipline. Do not respond when a claim has low reach and responding would introduce it to a wider audience; when you cannot yet offer a credible, sourced correction; or when a trusted third party (a clinician, the Ministry) is better placed to speak. Silence on a fringe claim is sometimes the correction.

The counter-message formula (truth sandwich)

FACT (clear, memorable) → MYTH (stated once) → FALLACY (name the manipulation) → FACT (repeat). Keep it short enough to forward on WhatsApp, in the audience's language, from a trusted human.

APPENDIX D

Campaign Planning Worksheet

One page to run before any SRHR campaign or activity. It walks the whole guide's logic in order — system, audience, frame, messenger, channel, disinformation, action, measurement.

1 · The system (Module 1)

Which gears — patriarchy, culture/religion, disinformation, backlash — hold this barrier in place? Which will our activity touch?

2 · The audience (Modules 3–5)

Exactly who? What do they already believe SRHR means, and what value do they hold most closely?

3 · The frame (Module 5)

What shared value do we open on (health / family / provision / confidence)? What is the one idea?

4 · The messenger (Modules 5–6)

Which trusted person carries it — Level 1, 2 or 3? Are they briefed and willing?

5 · The channel (Module 6)

Where does this audience actually spend attention (WhatsApp / radio / TikTok / baraza)? Language?

6 · Disinformation (Module 2)

What myth might collide with this message? Do we lead with the fact and prebunk the myth?

7 · The action (Module 3)

What dignified next step do we point to, and have we lowered the cost of taking it?

8 · Measurement (Module 7)

What realistic intermediate outcome will we watch, over what horizon?

APPENDIX E

Pre-Publication Message Checklist

Run every SRHR message through these checks before it goes out. If any answer is “no,” rework before publishing.

- ☐ It opens on a shared value (health, family, responsible choice) — not on rights language.
- ☐ It comes from, or quotes, a messenger this audience already trusts.
- ☐ If it addresses a myth, it leads with the fact and names the myth only once (truth sandwich).
- ☐ It says what SRHR is, not only what it is not — it does not repeat the opposition’s frame.
- ☐ It is in the right language (Kiswahili / Sheng) and channel for its audience, and it is forwardable.
- ☐ It is legally accurate (Article 43; the 2019/2022 rulings) where it touches the law.
- ☐ It ends by pointing to safe, accurate information or dignified, confidential services.
- ☐ It does not caricature older women, shame the audience, or amplify a myth to correct it.

Pinned beside a desk, this checklist is the fastest way to make the whole guide operational: it turns eight modules of evidence into eight questions anyone on the team can answer in two minutes.

APPENDIX F

Evidence Map — the headline number behind each module

A one-glance reference to the single figure that anchors each module, and where it comes from. Full citations are in the Sources list.

Module	Anchor figure	What it establishes
1 · The System	3.6% positive content	The information space the connected system has been left to fill.
2 · Disinformation	16.8M reached in 8 weeks	Disinformation moves at social speed; corrections crawl behind.
3 · Culture & Patriarchy	76% fear community judgment	The binding barriers are social and structural, not logistical.
4 · Mothers & Aunties	67% older-women influence	The most trusted channel is the highest-leverage intervention point.
5 · Reframing	8% link SRHR to rights	The audience starts at health and family, not rights.
6 · What Works	675,000 reached by faith-led FP	Trusted messengers and layered channels move real outcomes.
7 · Making It Routine	3.6% vs an always-on threat	Only standing habits can meet a permanent, connected challenge.

Figures drawn from the ImaarishaSRHR social listening, TIFA poll and research report, and the CHAK evaluation — see Sources.

APPENDIX G

Glossary of Frameworks & Terms

Continued-influence effect. The tendency of misinformation to keep shaping beliefs even after it has been corrected; the reason corrections must replace a myth with an alternative account, not just negate it.

Descriptive / injunctive norms. What people believe others actually do (descriptive) and approve of (injunctive); behaviour tracks these more than it tracks facts.

Diffusion of innovations. The theory that new ideas spread through social systems via trusted "opinion leaders" whose adoption gives others permission to follow.

Framing. The mental structure a message triggers with its first cues; the same fact framed as a right, a health benefit or a family duty reaches different people.

Gender continuum. The IGWG/USAID scale from gender-blind → exploitative → accommodating → transformative, used to locate how deeply an intervention addresses unequal power.

Infodemic. An overabundance of information, including false or misleading content, that spreads during a health event and undermines the response.

Inoculation / prebunking. Building resistance to manipulation by exposing people to a weakened form of it, and explaining the technique, before they meet the real thing.

Misnomerization. The Phase 1 term for how everyday naming narrows SRHR from a rights-and-dignity framework down to a narrow women's clinical service.

Narrative streams (S1–S7). The seven recurring frames the social-listening analysis used to organise Kenya's SRHR conversation; their documented interactions are the evidence that the barriers form one system.

Social-ecological model. The framework locating behaviour's drivers at individual, interpersonal, community/institutional and structural/policy levels.

Truth sandwich. A correction structured fact → myth (once) → fallacy → fact, so the audience leaves with the truth on top.

Values framing / sequencing. Leading with the values the audience already holds (health, family) and moving toward rights from there — "health is the entry point; rights is the destination."

The future of SRHR in Kenya will be determined by who gets to define the meaning of reproductive health in public and private life.

This guide is Community Media Trust's contribution to that definition. Respectfully. Relentlessly. Together.

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